

POSTING EMERGENCY SURGICAL PROCEDURE DURING ON CALL HOURS WITH CALL TEAM

SOP 112-066

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Service Line(s):
Operating Room

Signatory Authority:
Operative Care Line Executive

Effective Date:
August 29, 2025

Responsible Owner:
Operating Room Nurse Manager

Recertification Date:
August 29, 2030

1. PURPOSE AND AUTHORITY

a. To provide guidelines for posting emergency surgical procedures and adequate coverage of nursing personnel in the operating room after hours, weekends and holidays.

2. PROCEDURES

a. Surgery Resident/ Surgery Attending/ Transplant Coordinator

(1) Senior Resident performing the procedure will discuss the case with Surgical Staff on call.

(2) Notify the in-House Anesthesiology provider, pager # 281-567-1971 then call the Staff anesthesiologist to discuss the case. Staff Anesthesiologist telephone can be found on under desktop gold star icon =>HOU Shortcuts => MEDVAMC On Call => Anesthesia. Staff Anesthesiologist has 45 minutes from notification to arrival for emergency cases.

(3) If everybody concurs, the resident will complete an OR posting slip (VA-10-2370) or transplant coordinator will enter a posting note in CPRS and give it to the SICU Charge RN, then verify all required information:

- a) Post op destination
- b) Expected surgery start time
- c) Length of procedure
- d) Cell Saver/ perfusion (if needed)
- e) Intra op X-ray (If needed)
- f) Baseline CBC, BMP, Chest Xray, EKG, Type and screen.

g) Blood order and cross match if necessary.

(4) Make sure the consent is correct and signed.

(5) Make sure LST note is complete

(6) In case of delay or cancellation of posted emergency, Surgery Resident/Surgery Attending/ Transplant coordinator will notify Anesthesiology staff and the OR Nurses ASAP.

b. **SICU Staff / transplant coordinator**

(1) Call OR Nurses (2) and Surgical Tech (1) on OR call and if required, specialty nurse: CV, Neuro, and Liver. From Qgenda

a) Cell Phone

b) Pager

c) Home Phone

(2) Notify Perfusionist on call (if necessary).

(3) All calls will be logged in the logbook and timed. (The OR Emergency Crew Schedule is in the call book)

(4) If more OR Staff are needed, or specialty is needed, call the respective specialty on call team.

(5) If unable to contact the nurse on call, proceed and call the OR Nurse Manager.

c. **OR Nurses**

(1) **Getting a call for a case**

a) Inquire what surgery case is posted

b) Inquire time surgery starts

c) Inquire SICU to call other on-call nursing team members

d) Arrive 45 minutes or less after you receive the call

(2) **Arrival in the VA**

a) Get the following items from SICU

1. Posting Slip/ or CPRS transplant coordinator posting note
2. OR Call book
3. OR keys
4. Check with the SICU Charge Nurse if bed and /or staff are available to recover the patient.

(3) Before Surgery

- a) Post case in Vista (Instructions on page 4)
- b) Check consent, LST, attending surgeon note, H & P, blood status and other pertinent patient information
- c) Check for availability of Case Cart use back up
- d) Call SPS for Case Cart replacement, Ask SPS to Replace the Emergency case cart that is going to be used. (ext. 223032, 223145 or decontamination 223034) (Available 24/7)
- e) Secure blood from the blood bank if required
- f) Call perfusionist (On-call list is listed on the Front desk board) if needed
- g) Call in-patient pharmacy for cardioplegic solution if needed (ext. 224120)
- h) Call x-ray if needed (ext. 24516, 24518, 24514, 25592) (Pager *5 1217)
- i) Call CT Scan for angiography needs (ext. 24524) (Pager 713-841-0262)
- j) Receive and assess patient in holding area or ICU ward
- k) Forward the phone calls from Front desk to the OR room where you will perform surgery (Instructions on page 4)
- l) Call the PACU nurse on call if SICU is not able to recover. (on call list in OR book)

(4) After Surgery

- a) Call report to PACU/SICU RN at the end of the procedure and document the full name of the personnel that received the report.

- b) All permanent specimens need to be in the refrigerator after 3.30pm Monday to Friday, weekends, and on holidays with the required amount of formalin. Page * 5-2135 after 3.30pm for special handling of specimens.
- c) A large histopathology specimen (ex. extremity) is wrapped, secured, labeled and stored in the Specimen Room refrigerator.
- d) Return Blood/Cooler to blood bank.
- e) Ask the Operator for EMS on-call number.
- f) Call EMS and inform them of the OR room that needs cleaning.
- g) Fill up 24-hour sheet (VA Form 10-2913) from file cabinet B
 - 1. Pt. Last name, First name, Last 4 of SS#
 - 2. Surgery performed
 - 3. Resident surgeon
 - 4. Staff surgeon
 - 5. Attending code (A, B, C, D)
 - 6. Time in the OR room to end time (use military time for consistency)
- h) Fill up OCL Nursing Unit OT/ CT Time Log Sheet from file cabinet B (each sheet is good for one day only 24:00 - 24:00)
 - 1. Write SOR and date
 - 2. Nursing staff write the OT/ CT times
- i) Fill up OT/ CT book (applying highlighter on one line after the last name entry indicates that you are starting a new date)
- j) Forward back calls from the room to Front desk phone (instruction below)
- k) Bring OR Call Book and keys back to SICU and endorse SICU personnel

(5) Heart Transplant and LVAD/Cases

- a) The OR will activate an in-house team to work overtime (OT) 72 hours after any Heart Transplant/LVAD patients including high risk patients.

- b) OR Pool will be open to all from the hours of midnight to 5 am.

(6) Forwarding calls from front desk phone to another location.

- a) Lift handset of Front desk phone
- b) Press FWD key (located in the display screen)
- c) Dial Ext Number
- d) Hang-up
- e) Blinking light means you can forward calls

(7) Cancel forward calls

- a) Lift handset of Front desk phone
- b) Press FWD key
- c) Hang up
- d) Blinking light will disappear

(8) Posting and emergency case

- a) Secure posting slip or CPRS transplant coordinator posting note
- b) Go to Vista
- c) Type in SURG
- d) Type in Surgery Nursing
- e) Type in O
- f) Type in patient last name initial and last 4 of SS# and choose your patient
- g) Type in the Number of Enter New Surgical Case
- h) Follow promptly, important items to write on chart are:
 - 1. Select date of operation

2. Desired procedure date
3. Procedure name
4. Diagnosis
5. Surgeon name
6. Staff name
7. ICD code
8. CPT code in PO (Planned principal Procedure Code)
9. For Specialty refer to the following code:
 - a) Peripheral for Vascular
 - b) Cardio for Cardiothoracic
 - c) Neuro for Neuro
 - d) General for General
 - e) ENT for ENT
 - f) Plastic for Plastic
 - g) Ortho for Ortho
 - h) Ophthalmology for Ophthalmology
 - i) Urology for Urology
 - j) Gynecology for Gynecology**
 - k) Transplant
- i) Hit Shift 6 (^ sign) if you do not know the answer for whether surgery is related to combat or if surgery is due to Agent Orange. (The above question/s may or may not be asked.)
- j) By posting, the information will be carried over to ORC Monitor Board

3. DEFINITIONS

None

4. REFERENCES

None

6. REVIEW

Five-year review period at recertification. This SOP will be reviewed, at minimum at recertification, when there are changes to the governing document and any regulatory requirement for more frequent review.

7. RECERTIFICATION

This SOP is scheduled for recertification on or before the last working day of August 2030. In the event of contradiction with national policy, the national policy supersedes and controls.

8. SIGNATORY AUTHORITY

JENNIFER
SOLAK

Digitally signed by
JENNIFER SOLAK
Date: 2025.09.05
13:45:08 -05'00'

Jennifer Solak, MBAHM, BSN, CNOR
Operating Room Nurse Manager
Date Approved: August 29, 2025

SAMIR
AWAD

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Samir Awad, MD, MPH
Operative Care Line Executive
Chief of Surgery
Date Approved: August 29,
2025

NOTE: The signature remains valid until rescinded by an appropriate administrative action.

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